INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Durvalumab 10 mg/kg every 14 days

Understand your treatment. Know what to expect. Keep your plan to hand.

Displayed calendar
14 days
See the active treatment phase

Treatment stages
1
Explained in the treatment reference

Regimen source
NCCP 00576
Version 5

Day 1 · every 14 days

Treatment stages

01 **Durvalumab every 14 days after completed concurrent chemoradiotherapy (CRT)**
Consolidation

Your team confirms the active stage and dates. The full treatment guide explains each step.

Inside your handbook

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My name

Hospital / oncology team

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

CHAPTER 01

Understanding your treatment

IN THIS CHAPTER

Why this treatment is offered, what its medicines do, and how the agreed plan fits your care.

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Contact the oncology team promptly for new or worsening cough, breathlessness or chest pain; diarrhoea, blood in stools or significant abdominal pain; yellow skin/eyes or dark urine; reduced urine; a spreading or blistering rash; unusual weakness, severe headache, dizziness, marked thirst or passing much more urine. Severe breathing difficulty, collapse, confusion, seizures, rapidly worsening weakness or difficulty swallowing need emergency assessment. Do not wait for the next infusion.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

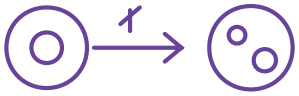
Your immunotherapy medicine

Durvalumab is given after definitive chemoradiotherapy to reduce the chance of lung cancer returning or worsening and improve the chance of longer survival. This is part of a treatment pathway that aims for long-term control and possible cure; it cannot guarantee cure.

PD-L1 IMMUNE CHECKPOINT INHIBITOR

Durvalumab

Intravenous infusion every 14 days



Blocks PD-L1 to support immune recognition of cancer

Durvalumab blocks PD-L1, a protein that can reduce the immune system's ability to attack cancer cells. Removing this signal can help immune cells act against cancer, but the immune system can also inflame healthy organs.

PD-L1 can dampen an immune response → Durvalumab blocks the PD-L1 signal → Immune cells can act against cancer cells

What to watch for

Report new symptoms early and carry the immunotherapy alert card. Do not wait for the next infusion.

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

Why this treatment may be chosen

In PACIFIC, durvalumab reduced cancer worsening and improved overall survival compared with placebo after chemoradiotherapy. The results describe groups of trial participants and do not predict what will happen to one person.

CHAPTER 02

Your schedule, tests and appointments

IN THIS CHAPTER

Your treatment stages, checks and practical preparations, from before treatment through follow-up.

Chemoradiotherapy first, then durvalumab

Durvalumab 10 mg/kg every 14 days

14-day view



This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

CONSOLIDATION

Durvalumab every 14 days after completed concurrent chemoradiotherapy (CRT)

After completed concurrent CRT | **Every 14 days** | **Up to 12 months (26 cycles)**

Durvalumab is infused on Day 1 of each 14-day cycle. The course lasts up to 12 months (26 cycles), and can finish earlier if the cancer progresses or side effects make continuation unsuitable. The first dose should start within 42 days after chemoradiotherapy finishes.

The earlier chemotherapy and radiation course has its own prescribing and support plan. Durvalumab is the consolidation phase covered here.

BASELINE → TREATMENT → FOLLOW-UP

Checks across your durvalumab course

Relative planning stages, not booked appointments or equal time intervals. Follow the team's actual blood-test and scan schedule; extra endocrine, lung or heart tests depend on symptoms and findings.

Check	Before durvalumab	First infusion	Later cycles	Between visits	After completion / a hold
Treatment sequence	Finish concurrent chemotherapy + radiotherapy	Start within the source window	Day 1 every 14 days	No durvalumab tablets at home	Up to 12 months / 26 cycles; review any restart
Blood count / kidney / liver	Baseline tests	Review available results	Regular tests under the team plan	Extra tests for symptoms or abnormalities	Follow abnormalities until resolved / stable
Glucose / thyroid / hormones	Baseline glucose and thyroid tests	Review symptoms and medicines	Regular glucose and thyroid tests	Targeted hormone tests when indicated	Ongoing replacement and follow-up if needed

Check	Before durvalumab	First infusion	Later cycles	Between visits	After completion / a hold
Lung and other immune symptoms	Review CRT recovery and baseline breathing	Symptom check	Ask before each infusion	Report changes promptly	Delayed immune effects still need assessment
Hepatitis / tumour / scans	Hepatitis B/C screen; validated PD-L1	Confirm stage and no progression	Scans as the consultant plans	New symptoms may need earlier assessment	Agree follow-up and contact arrangements

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00576 v5 · official regimen · EMA Imfinzi product information · sections 4.2–4.6, 5.1

THE WHOLE TREATMENT JOURNEY

Durvalumab 10 mg/kg every 14 days: treatment and checks

Stage III non-small-cell lung cancer (NSCLC) · consolidation after concurrent chemoradiotherapy

Durvalumab is infused on Day 1 of each 14-day cycle. The course lasts up to 12 months (26 cycles), and can finish earlier if the cancer progresses or side effects make continuation unsuitable. The first dose should start within 42 days after chemoradiotherapy finishes.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before durvalumab	First infusion	Later cycles	Between visits	After completion / a hold
Treatment sequence NCCP 00576	Finish concurrent chemotherapy + radiotherapy	Start within the source window	Day 1 every 14 days	No durvalumab tablets at home	Up to 12 months / 26 cycles; review any restart
Blood count / kidney / liver NCCP 00576	Baseline tests	Review available results	Regular tests under the team plan	Extra tests for symptoms or abnormalities	Follow abnormalities until resolved / stable
Glucose / thyroid / hormones NCCP 00576 · SMPC	Baseline glucose and thyroid tests	Review symptoms and medicines	Regular glucose and thyroid tests	Targeted hormone tests when indicated	Ongoing replacement and follow-up if needed

Care / check	Before durvalumab	First infusion	Later cycles	Between visits	After completion / a hold
Lung and other immune symptoms NCCP 00576 · SMPC	Review CRT recovery and baseline breathing	Symptom check	Ask before each infusion	Report changes promptly	Delayed immune effects still need assessment
Hepatitis / tumour / scans NCCP 00576	Hepatitis B/C screen; validated PD-L1	Confirm stage and no progression	Scans under the agreed follow-up plan	New symptoms may need earlier assessment	Agree follow-up and contact arrangements

What each check is for

1 **Finish CRT and confirm suitability**

Before the first durvalumab dose

Chemotherapy and radiation are concurrent first; durvalumab follows after completion.

Ask / act: Review the actual treatment sequence, recovery and baseline tests.

2 **Recognise lung and endocrine symptoms**

Throughout the course, holds and follow-up

Immune inflammation and radiation effects can overlap; hormone problems can develop with subtle symptoms.

Ask / act: Report changes promptly and keep the alert card available.

3 **Complete, pause or stop safely**

At toxicity review or the maximum course

The course is capped at 12 months (26 cycles), with earlier stopping if needed.

Ask / act: Agree follow-up, hormone replacement if needed and how to obtain urgent advice after completion.

4

Agree follow-up imaging

After CRT, during treatment and after completion

Look for recurrence when further intervention would be appropriate.

Ask / act: Follow your team’s scan plan and report new symptoms without waiting for a routine scan.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Practical treatment planning

Agree the arrangements with your oncology team.

STEP 1

Two different parts of treatment

Platinum chemotherapy and radiation are given together first. Durvalumab follows after that course has finished and the team has confirmed that the cancer has not progressed.

STEP 2

Infusion visits

The usual reference dose is 10 mg/kg, given into a vein over about 60 minutes every 14 days. Allow extra time for the clinical review, tests and preparation.

STEP 3

Bloods, scans and transport

Confirm where blood tests are taken, who reviews them and the planned clinic/scan appointments. Allow extra time at the treatment unit and arrange help with travel if fatigue or symptoms make it difficult.

STEP 4

A pause or early finish

Treatment can be paused to investigate symptoms or treat an immune reaction. A missed dose does not automatically extend the course beyond the planned maximum. Restart decisions belong to the consultant team.

Tests and symptom review

Before starting: blood count, kidney and liver tests, blood sugar, thyroid function, hepatitis B/C screening and the tumour PD-L1 result. Blood count, kidney/liver tests, blood sugar and thyroid tests continue regularly during treatment. The team sets the timing of blood tests and scans and adds tests when symptoms or results need checking.

Breathing after chest radiotherapy

Radiation inflammation, immune inflammation, infection and cancer can cause similar symptoms. Report new or worsening breathing symptoms promptly so the team can assess the cause. Do not assume a new cough is only an old radiotherapy effect.

Hormone monitoring

Thyroid, adrenal or diabetes problems may need hormone replacement or insulin, sometimes long term. Keep taking prescribed medicines and attend blood tests even when you feel well. Sudden severe weakness, vomiting, faintness, unusual thirst or drowsiness need urgent advice.



Scans and targeted tests

Scans follow your consultant’s treatment plan. Extra lung, heart, hormone or other tests depend on symptoms and findings; the guide does not set an automatic repeat heart-scan calendar.

CHAPTER 03

Symptoms and supportive care

IN THIS CHAPTER

Recognising changes, knowing when to call, and using your prescribed supportive-care plan.

Day 1 → Day 14: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Treatment or check	Timing in this view
Durvalumab · Day 1	 **Day 1** intravenous (IV) infusion after review and planned tests
Symptoms and planned tests	 **Days 1–14** Report new symptoms promptly; follow the team's test schedule

Sequence: Concurrent chemotherapy and radiotherapy come first; durvalumab follows after completion.

Start window: The first dose should commence within the source's 42-day window after CRT.

Course limit: Continue no longer than 12 months (26 cycles), with earlier stopping for progression or unacceptable toxicity.

After treatment: Monitoring and reporting continue during holds and after treatment ends.

> **Why these bands stay broad:** This example shows one 14-day durvalumab cycle after concurrent CRT has finished. It is not a dated appointment calendar; the actual prescription and clinical decisions determine visits. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00576 v5 · official regimen ↗

> **There are no guaranteed "safe days".** Use the current prescription and your oncology team's emergency plan. This educational guide does not determine whether a dose can be given.

Between-treatment notes

Day 1: Infusion visit

Durvalumab 10 mg/kg reference dose by IV infusion over about 60 minutes after clinical review and planned tests. Report new symptoms and any treatment changes before the infusion.

Day 2–14: Between infusion visits

No durvalumab tablets are taken at home. Follow the agreed blood-test, supportive-medicine and symptom plan. Report new or worsening symptoms promptly; do not wait for the next cycle.

Support around the medicines

1 Immune side effects and the alert card

Immune side effects may start during treatment or after the last dose. Keep the immunotherapy alert card and tell every treating clinician that you have received durvalumab, even during a treatment break or after finishing.

2 **Support medicines**

Durvalumab alone has minimal sickness risk and no routine premedication is specified. Use anti-sickness or other medicines only as prescribed. Steroids for an immune reaction are a treatment plan supervised by the team, not routine premedication; do not start, reduce or stop them yourself.

3 **Hormone replacement and blood tests**

4 **Pregnancy, fertility and breastfeeding**

Discuss pregnancy plans, fertility and breastfeeding before treatment. Effective contraception is advised during durvalumab and for at least three months after the last dose. Durvalumab is not recommended during pregnancy. The team must help decide whether breastfeeding or durvalumab should be stopped or avoided, weighing the benefits and risks.

Treatment and its aim

Who this guide covers

Consolidation for unresectable stage III non-small-cell lung cancer after concurrent platinum chemotherapy and radiotherapy, with no cancer progression and tumour PD-L1 of at least 1%.

Infusion and personal decision

The usual reference dose is 10 mg/kg, given into a vein over about 60 minutes every 14 days. Allow extra time for the clinical review, tests and preparation. PACIFIC directly studied durvalumab every two weeks after concurrent chemoradiotherapy. This guide uses that interval.

Support and urgent symptoms

Support medicines

Hormone replacement and blood tests

Scans and follow-up

Follow-up continues after treatment finishes.

Agree your scan plan

Published expert guidance suggests chest/abdomen computed tomography (CT) with contrast if suitable every 3 months in year 1 after chemoradiotherapy, at least every 6 months in year 2 and at least yearly in years 3–5. Your team agrees your actual plan; this is supplemental guidance, not a fixed NCCP requirement. New symptoms may need earlier assessment. Routine brain scans are not recommended without symptoms after baseline staging.

Questions worth asking

Is this more chemotherapy?

Durvalumab is an immunotherapy infusion. It follows the completed chemotherapy and radiation course and works by changing immune signalling.

Why report a cough after chest radiotherapy?

Radiation effects, immune inflammation, infection and cancer can look alike. The team needs to assess new or worsening symptoms rather than assume their cause.

Will I need routine steroids before each infusion?

No routine premedication is specified for this regimen. Steroids may be prescribed to treat particular immune side effects, with a supervised dose and taper.

Try explaining the plan back: What is the sequence of chemoradiotherapy and durvalumab, how often are your infusions, what is the planned maximum course, and which symptoms mean you should contact the team immediately? Who should see your immunotherapy alert card?

Durvalumab

- Inflammation of the lungs, bowel, liver, kidneys or skin
- Thyroid, adrenal, pituitary or blood-sugar problems that may need long-term replacement treatment
- Rare serious inflammation of the heart, muscles or nerves
- Infusion reactions, fatigue and infections; effects can appear after treatment ends

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Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Durvalumab consolidation · every 14 days

Active medicines: Durvalumab

Emetogenic risk: Minimal

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

Additional phase-specific support

- No routine antiemetic prophylaxis for durvalumab alone; individual/local-policy rescue treatment if needed.
- No routine premedication specified; consider a reviewed plan after an infusion reaction.
- Steroids or immunosuppressants for immune toxicity require an individual consultant-led treatment and taper plan, not automatic prophylaxis.
- Provide immunotherapy alert card, emergency contacts, pulmonary-symptom advice and regular glucose/thyroid/organ-function testing.
- The preceding chemoradiotherapy has a separate supportive-care plan; do not carry its antiemetic steroids forward automatically.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

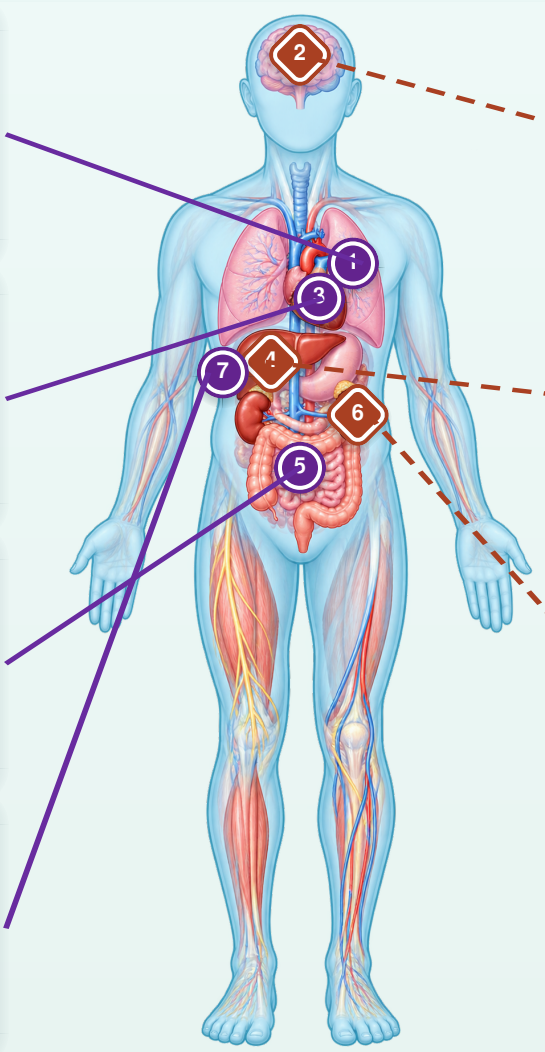
Durvalumab 10 mg/kg every 14 days

Durvalumab is infused on Day 1 of each 14-day cycle. The course lasts up to 12 months (26 cycles), and can finish earlier if the cancer progresses or side effects make continuation unsuitable. The first dose should start within 42 days after chemoradiotherapy finishes.

Source reconciled; independent clinical, pharmacy and patient/PPI review pending. Selected effects; not a complete medicine leaflet.

A picture of side effects

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.



1 Lungs / breathing

Durvalumab

Report new or worsening cough, breathlessness or chest pain promptly, including after finishing radiotherapy.

2 Hormones / head / nerves

Durvalumab

Severe headache, dizziness, confusion, unusual weakness or thirst need prompt assessment; collapse or seizures need emergency help.

3 Heart / muscles

Durvalumab

Chest pain, palpitations, marked weakness or trouble swallowing can signal serious inflammation; seek urgent help.

4 Liver

Durvalumab

Report yellow eyes/skin, dark urine or significant upper abdominal pain; blood tests may detect changes before symptoms.

5 Bowel

Durvalumab

Report new diarrhoea, blood in stools or significant abdominal pain; do not assume this is ordinary stomach upset.

6 Kidneys / urine

Durvalumab

Report much less urine, swelling or a change in urine; attend kidney blood tests.

7 Skin

Durvalumab

Report a spreading rash. Blistering, peeling or mouth/eye sores need urgent assessment.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

These are examples of immune side effects, not a diagnosis or a complete list. Symptoms may have other causes, and blood tests can detect inflammation before you feel it.

> Contact the oncology team promptly for new or worsening cough, breathlessness or chest pain; diarrhoea, blood in stools or significant abdominal pain; yellow skin/eyes or dark urine; reduced urine; a spreading or blistering rash; unusual weakness, severe headache, dizziness, marked thirst or passing much more urine. Severe breathing difficulty, collapse, confusion, seizures, rapidly worsening weakness or difficulty swallowing need emergency assessment. Do not wait for the next infusion.

CHAPTER 04

Sources and further information

IN THIS CHAPTER

The references behind this information and addresses you can use to find further help.

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **National Cancer Control Programme (NCCP) 00576 v5 · official regimen**
   https://healthservice.hse.ie/documents/6888/576_Durvalumab.pdf
2. **EMA Imfinzi product information · sections 4.2–4.6, 5.1**
   https://www.ema.europa.eu/en/documents/product-information/imfinzi-epar-product-information_en.pdf
3. **Ko et al. 2023 PACIFIC-based expert follow-up imaging recommendations (supplemental; not NCCP-mandated)**
   https://pmc.ncbi.nlm.nih.gov/articles/PMC10137068/
4. **Spigel et al. PACIFIC five-year update, JCO 2022; DOI 10.1200/JCO.21.01308**
   https://ascopubs.org/doi/10.1200/JCO.21.01308
5. **EMA durvalumab fixed-dose pharmacokinetic bridging assessment**
   https://www.ema.europa.eu/en/documents/variation-report/imfinzi-h-c-4771-ii-0014-g-epar-assessment-report-variation_en.pdf
6. **HSE · chemotherapy safety and when to contact your team**
   https://www2.hse.ie/conditions/chemotherapy/side-effects/